

# CodePilot

*AI-Assisted ICD-10-CA Coding — Technical Build & Deployment*  
*Full Stack Data Science Systems*  
*Group 7*

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# Agenda

1. Problem
2. Significance
3. Literature Review
4. Methodology — ML Canvas
5. Model Benchmarking
6. Model Deployment
7. Challenges & Issues Encountered
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# Problem

Manually matching a clinical note to the correct diagnosis code is slow and error-prone.

- The ICD-10-CA catalog (Ontario Health / CIHI) contains 15,573 codes — too many to search manually per chart.
- Coding accuracy directly affects Ontario's HBAM funding model, not just billing — errors have real financial consequences.
- The medical coding workforce is shrinking and aging, increasing per-chart turnaround time.

# Why This Problem

**30%**

national coder shortage

**50+**

average coder age

**15,573**

ICD-10-CA codes to search

*This is a real, present-tense operational problem — not a hypothetical future one. It affects funding accuracy, staff workload, and turnaround time today.*

# Literature Review

## Past Available Projects

- Nuance / Microsoft DAX — ambient documentation + coding
- 3M 360 Encompass — enterprise coding & CDI platform
- Fathom Health, CodaMetrix — autonomous AI coding startups

## Similar Academic Approaches

- CNN/BERT-based ICD classifiers trained on MIMIC-III/IV
- ClinicalBERT for readmission & coding prediction
- Multi-label classification with 40K+ labeled notes

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## Drawbacks We Identified

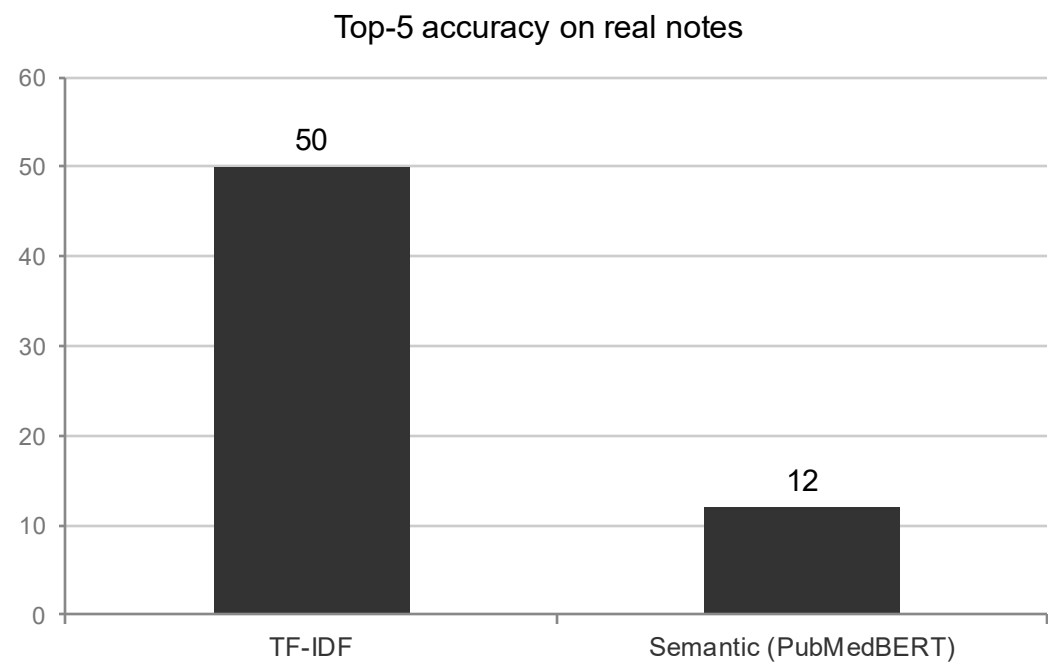
- Enterprise platforms priced for large multi-site health systems, not a single mid-size hospital
- Most academic work trains on MIMIC (ICD-9 or US ICD-10-CM) — not Canada's ICD-10-CA classification
- Classifier-based approaches need thousands of labeled (note, code) pairs — infeasible for us to assemble at project timeline

# ML Canvas

|                                                                                                                                   |                                                                                                                            |                                                                                                           |
|-----------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------|
| <b>Value Proposition</b><br><br>Coders get top-5 ranked ICD-10-CA suggestions per note instead of searching 15,573 codes manually | <b>Data Sources</b><br><br>Official ICD-10-CA catalog (Ontario Health/CIHI) + 16 hand-verified real notes (MTSamples, CCO) | <b>Prediction Task</b><br><br>Ranked retrieval: note text -> top-k most similar code descriptions         |
| <b>Decisions</b><br><br>Coder confirms, overrides, or requests more candidates — model never auto-submits                         | <b>Building Models</b><br><br>TF-IDF baseline + pretrained PubMedBERT embeddings — no training from scratch                | <b>Offline Evaluation</b><br><br>Top-1/Top-5/category-match accuracy + latency, benchmarked on real notes |
| <b>Features</b><br><br>Free-text note only (current scope) — no structured EHR fields yet                                         | <b>Live Evaluation (planned)</b><br><br>Top-5 acceptance rate, override rate, time-per-chart vs. baseline                  | <b>Risks</b><br><br>Multi-comorbidity notes, semantic model catalog bias — both tested & documented       |

# Model Benchmarking

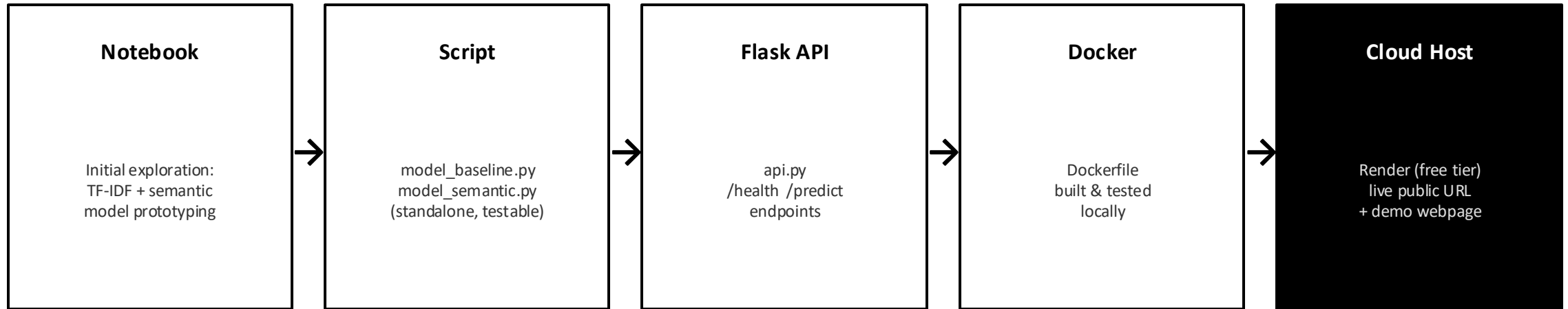
Two pretrained approaches, benchmarked on 16 real, hand-verified clinical notes against the ICD-10-CA catalog



| Metric         | TF-IDF  | Semantic |
|----------------|---------|----------|
| Top-1 accuracy | 12%     | 0%       |
| Top-5 accuracy | 50%     | 12%      |
| Category match | 25%     | 25%      |
| Avg latency    | 0.75 ms | 17.3 ms  |

Key finding: the simpler keyword model outperformed the pretrained semantic model — root-caused to catalog composition (9.4% of ICD-10-CA is obstetric codes with repetitive phrasing), not just "the model is worse."

# Model Deployment



**Live:** <https://icd10-coder.onrender.com>

Source: <https://github.com/Vikashkhatri/icd10-coder>



# Challenges & Issues Encountered

## Apple Silicon numerical bug

Semantic model produced NaN/Inf values on MPS backend — fixed by forcing CPU + sanitizing embeddings

## MIMIC-III demo has no notes

PhysioNet strips all clinical text from the demo tier by design — pivoted to public MTSamples corpus

## Wrong code system initially

Prototype started on US ICD-10-CM; corrected to the real ICD-10-CA standard used by Ontario hospitals

## Semantic model catalog bias

Unexplained drift toward obstetric codes — root-caused to catalog composition, not a simple bug

## Multi-comorbidity notes

Neither whole-note nor sentence-split querying reliably handles 3+ conditions in one note — documented as an open limitation

## No 500+ real labeled dataset

Investigated MIMIC-IV, Kaggle, CodiEsp — concluded credentialed access is the only trustworthy path at scale

# Demo

*Live, public, deployed right now*

icd10-coder.onrender.com

**Clinical note:**

"45 year old male with type 2 diabetes mellitus, poorly controlled, presenting for routine follow-up."

**Suggested ICD-10-CA codes:**

**E11.33** Type 2 diabetes with other retinopathy

**E10.33** Type 1 diabetes with other retinopathy

**E11.41** Type 2 diabetes with polyneuropathy

Model: TF-IDF · Catalog: ICD-10-CA (15,573 codes) · Deployed on Render (free tier)

# Q & A

`github.com/Vikashkhatri/icd10-coder` · `icd10-coder.onrender.com`